June 16, 2025

The Honorable Mehmet Oz
Administrator
Centers for Medicare & Medicaid Services
U.S. Department of Health and Human Services
200 Independence Avenue, SW
Washington, DC 20201

Thomas Keane
Assistant Secretary for Technology Policy
National Coordinator for Health Information Technology
U.S. Department of Health and Human Services
330 C Street, SW
Washington, DC 20201

RE: The Diabetes Technology Access Coalition's Response to *Request for Information; Health Technology Ecosystem* (CMS-0042-NC)

Dear Administrator Oz and Assistant Secretary Keane,

The Diabetes Technology Access Coalition (DTAC) appreciates the opportunity to provide comments to the Centers for Medicare & Medicaid Services' (CMS or the "agency") and the Office of the National Coordinator for Health Information Technology's (ONC) Request for Information (RFI) on the Health Technology Ecosystem. Collectively, DTAC members represent millions of Americans with diabetes, health care professionals who treat them, and major manufacturers that develop diabetes therapies, equipment, and supplies. Thus, our coalition represents those who manufacture and develop diabetes technology, the health care professionals who rely on this technology to best treat their patients, and the patients who benefit from the technologies.

DTAC's primary goal is to remove barriers to access for critical diabetes technologies, including continuous glucose monitors (CGMs) and insulin pumps. Additionally, all insulin pumps currently available in the U.S. pair with a CGM, and with a software or algorithm, can control the flow of insulin provided based on the CGM readings. This combination of technologies, called an automated insulin delivery (AID) system, as well as CGMs and insulin pumps individually, can all be considered "digital tools" and "digital health products" as defined by CMS in this RFI, as they leverage various forms of software to operationalize their core functionalities, which in turn, support the health needs of individuals with diabetes. All of these technologies are part of the diabetes standards of care as articulated by the American Diabetes Association (ADA) and the American Association of Clinical Endocrinology (AACE) as they have been shown to effectively manage an individual's type 1 or type 2 diabetes.[1]

While DTAC has made strides in expanding coverage of diabetes technologies, innovation has outpaced the federal government's bureaucracy. Thus, we are pleased to see that CMS is interested in garnering stakeholder feedback in how to improve interest in and access to critical diabetes technologies and offer the following recommendations.

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[1] *Standards of Care in Diabetes – 2025*, 48 Diabetes Care S1 (2025), https://diabetesjournals.org/care/issue/48/Supplement_1; George Grunberger, et. al., *American Association of Clinical Endocrinology Clinical Practice Guideline: The Use of Advanced Technology in the Management of Persons With Diabetes Mellitus*, 27 Endocrine Practice 505 (2021), https://www.endocrinepractice.org/action/showPdf?pii=S1530-891X%2821%2900165-8; Lawrence Blonde, et. al., *American Association of Clinical Endocrinology Clinical Practice Guideline: Developing a Diabetes Mellitus Comprehensive Care Plan—2022 Update*, 28 Endocrine Practice P923 (2022), https://www.endocrinepractice.org/article/S1530-891X(22)00576-6/fulltext.

Improving Interest in Digital Health Products, Including Diabetes Technologies (PC-5)

Individuals with diabetes uniquely experience their disease and therefore greatly benefit from individualized care as jointly determined by the individual and the health care professional. Thus, to improve interest in digital health products for individuals with diabetes, individuals who would benefit from glucose control and management technologies must have ready access to a comprehensive and flexible set of treatment options. This includes ensuring that the Medicare program provides access to all forms of life-sustaining diabetes technologies, as they span different functions and can, but not necessarily, be used in conjunction with each other.

Unfortunately, CMS's process for updating coverage criteria, especially for National Coverage Determinations (NCDs), presents troubling issues that hinder interest in and access to digital health products. For example, the current NCD for insulin pumps (NCD 280.14) is based on outdated and dangerous coverage criteria that is 20 years out of date, inconsistent with commercial insurance coverage criteria, and misaligned with coverage for CGMs. Recognizing this, in February 2022, a group of leading diabetes clinicians requested that CMS update its coverage criteria for insulin pumps so they align with the current clinical evidence and standards of care. The agency – despite this request being submitted more than three years ago – has still not acted on this request. In the intervening three years, more developments have emerged that further support the reconsideration request, such as a randomized control trial (RCT) demonstrating that the use of an AID system was associated with improved clinical benchmarks compared to only using a CGM among adults with insulin-treated type 2 diabetes,[2] and the Food and Drug Administration (FDA) approved AID systems for the type 2 diabetes population.[3] The longer CMS fails to act on this request, the more divergent the coverage criteria for insulin pumps will become from the latest clinical evidence, access to insulin pumps and AID systems will stagnate despite growing interest in these technologies.

The Local Coverage Determination (LCD) process handled by the Medicare Administrative Contractors (MACs) shows a stark difference with the NCD process. For example, coverage criteria for CGMs are primarily handled by the Durable Medical Equipment (DME) MACs, who handle reconsideration requests in a much more expedient fashion. In September 2021, a group of clinicians submitted a CGM LCD reconsideration request to align coverage with the evidence and standards of care at the time, and in October 2022, the DME MACs released a proposed CGM LCD, which was finalized in April 2023. Likewise, in February 2025, a group of clinicians submitted a CGM LCD reconsideration request as recent research (including RCTs and real-world evidence) demonstrated that individuals with type 2 diabetes who are not on insulin, but use a CGM, achieve outcomes equal to or better than those with type 2 diabetes who use insulin.[4] Accordingly, the ADA updated its 2025 Standards of Care on CGM use for non-insulin-treated individuals with diabetes.[5] In the last three years, the CGM LCD could be updated twice before the insulin pump NCD is updated.

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[2] Yogish Kudva, et. al., *A Randomized Trial of Automated Insulin Delivery in Type 2 Diabetes*, New England Journal of Medicine (Mar. 19, 2025), https://www.nejm.org/doi/full/10.1056/NEJMoa2415948.

[3] *FDA Clears First Device to Enable Automated Insulin Dosing for Individuals with Type 2 Diabetes*, Food and Drug Administration (Aug. 26, 2024), https://www.fda.gov/news-events/press-announcements/fda-clears-first-device-enable-automated-insulin-dosing-individuals-type-2-diabetes.

[4] Gregory Norman, et. al., *Initiating continuous glucose monitoring is associated with improvements in glycemic control and reduced health care resource utilization for people with diabetes in a large US-insured population: A real-world evidence study*, 31 J. of Managed Care & Specialty Pharm. 15 (Jan. 2025), https://www.jmcp.org/doi/10.18553/jmcp.2024.24255; Andrew Karter, et. al., *Association of Real-time Continuous Glucose Monitoring With Glycemic Control and Acute Metabolic Events Among Patients With Insulin-Treated Diabetes*, 325 J. of Am. Med. Assoc. 2273 (June 2021), https://jamanetwork.com/journals/jama/fullarticle/2780594.

[5] *Standards of Care in Diabetes – 2025*, 48 Diabetes Care S1 (2025), https://diabetesjournals.org/care/issue/48/Supplement_1.

The stark difference between these two processes emphasizes the need to reform the NCD process. Beneficiaries continue to face insulin pump access barriers that do not align with current clinical evidence and standards of care. Timeliness is critical to promoting access to digital technologies, especially considering the rapid innovation and quickly evolving evidence base/standards of care in the diabetes space. Additionally, the coverage criteria for insulin pumps continue to diverge from the coverage criteria for CGMs, which is especially concerning given that AID systems are now part of the standard of care for individuals with diabetes. We thus urge the agency to reform its NCD processes and to quickly act on the NCD reconsideration request for insulin pumps to ensure beneficiaries can access interventions that are based on quickly evolving research and standards of care. Further, we hope that the DME MACs will act in the same expedient fashion they have done in the past in issuing a proposed CGM LCD that, consistent with the clinical evidence and standards of care, ensure all Medicare beneficiaries who would benefit from CGM use have ready access to this life-sustaining technology.

Finally, we also express our concern with CMS reviewing or approving digital health products outside of the coverage determination process. Coverage for all items and services in the Medicare system is guided by whether the item or service is "reasonable or necessary . . . for the diagnosis or treatment of illness or injury or to improve the functioning of a malformed body member."[6] Should CMS create an additional approval process, we are concerned this would create additional burdens and barriers for beneficiaries in accessing digital health tools or services. Unless doing so would streamline coverage decisions, we urge CMS not to pursue this approach.

Making Digital Health Products, Such as AID Systems, More Accessible (PC-6)

As new and innovative diabetes technologies arrive to the market and transform diabetes care, beneficiary education plays a key role in promoting access. Accordingly, we offer two primary suggestions related to diabetes technologies: (1) improving the diabetes self-management training (DSMT) benefit; and (2) ensuring beneficiaries can access educational resources for AID systems and insulin pumps.

DSMT is a proven and effective intervention that teaches individuals how to use diabetes technologies, administer medications, and implement healthy lifestyle changes that allow them to better manage their diabetes. Accordingly, the ADA, in its 2025 Standards of Care, recommends that "[a]ll individuals with diabetes should be advised to participate in developmentally and culturally appropriate diabetes self-management education and support (DSMES)." Given the demonstrated effectiveness of this intervention, any unnecessary barriers to DSMT must be removed.[7]

Individuals continue to face numerous barriers in accessing DSMT care, which in turn can hinder access to diabetes technologies, as individuals often rely on DSMT in the set-up and training to use these technologies. First, we wish to re-emphasize that diabetes is a highly personalized disease and that expanded access to a wide array of diabetes care is necessary not just for technologies, but also for diabetes education. However, regulations place rigid requirements on DSMT, such as requiring that the overwhelming majority of the initial DSMT benefit be given in a group setting, disallowing DSMT and medical nutrition therapy (MNT) to be delivered on the same day, and capping initial trainings at up to ten hours of DSMT within a continuous 12-month period and in subsequent years, receive up to two hours of DSMT. These requirements fail to provide the needed flexibility that should be afforded to individuals with diabetes, considering that some individuals might benefit from receiving their DSMT in a different modality (*i.e.*, a more personalized experience), or might need more DSMT hours throughout their lives to manage this lifelong disease.

[6] 42 USC 1395y(a)(1)(A).

[7] For regulatory citations that impose barriers to the DSMT benefit, we urge CMS and ONC to refer and review our response to CMS's RFI entitled *Unleashing Prosperity Through Deregulation of the Medicare Program Request for Information*.

Additionally, CMS's requirements on referral orders for DSMT (which require the provider to provide onerous details and duplicative information) and on program accreditation (which imposes administrative burdens that prevent new DSMT providers and existing programs to grow) create unnecessary barriers to accessing the DSMT benefit. We urge CMS to provide as much flexibility as possible to ensure all individuals can receive DSMT in the modality and at the frequency that both the beneficiary and their provider agree is best for the beneficiary's needs.

Additionally, while there is a payment mechanism for CGM set-up and training, there is no separate acknowledgement or reimbursement for providers who perform insulin pump set-up and training. Thus, individuals with diabetes must use their limited and front-loaded DSMT hours to obtain insulin pump training or providers must furnish this training without reimbursement. Eventually, as AID systems become more prevalent, similar codes for set-up and training will be needed for the algorithm component of these advanced systems, which is separate from and in addition to the use of the underlying CGM and pump technologies alone. Without proper education on these technologies, beneficiary interest in these products could remain stagnant thereby suppressing beneficiary access in diabetes technologies. Therefore, DTAC urges the agency to ensure that DSMT and other diabetes care and education services are as flexible as necessary to support a wide array of diabetes-related training and educational services spanning the multiple technologies available today and in the future.

Encouraging Providers to Leverage Digital Diabetes Health Products (PR-1)

CGMs can automatically transmit patients' glycemic information to their providers for analysis via remote monitoring, which has been shown to improve glycemic control.[8] Despite the evidence base for remote CGM monitoring, not all providers know how to use and leverage current technologies that facilitate this type of care. Accordingly, CMS needs to ensure that providers receive appropriate education on the availability of remote monitoring of glycemic levels and how to properly use and leverage data that is electronically delivered to them. Provider education on remote monitoring of glycemic control can be especially beneficial in rural areas. Unfortunately, rural Americans utilize CGMs at significantly lower rates than their urban counterparts.[9] Through educational resources, CMS can increase provider knowledge, and in turn, provider interest, in utilizing remote monitoring of CGM data. Additionally, given the disparities in CGM use based on geography, we urge the agency to collaborate with diabetes stakeholders to ensure equal access, across all geographies, to all covered diabetes technologies.

Stimulating Developer Interest in the Software or Algorithm of AID Systems (TD-1)

To ensure continued innovation and patient access to digital health products, CMS needs to ensure that it covers and values the software or algorithm that drives an AID system appropriately. We urge CMS to take proactive steps to support innovation by classifying the software or algorithm in AID systems as a supply to DME (*i.e.*, CGM or insulin pump) in order to maintain developer interest in AID system innovation.

As noted earlier, AID systems are driven by three equally important parts: a CGM, an insulin pump, and a software or algorithm that serves as the "brain" of the system. FDA has recognized the status of AID system algorithms as independent of CGMs and insulin pumps and has created the device category of

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[8] *See, e.g.*, Ronald Dixon, et. al., *A Virtual Type 2 Diabetes Clinic Using Continuous Glucose Monitoring and Endocrinology Visits*, Journal of Diabetes Science and Technology (Nov. 25, 2019), https://journals.sagepub.com/doi/full/10.1177/1932296819888662; Stephanie Crossen, et. al., *Remote glucose monitoring is feasible for patients and providers using a commercially available population health platform*, Frontiers in Endocrinology (Feb. 1, 2023), https://www.frontiersin.org/journals/endocrinology/articles/10.3389/fendo.2023.1063290/full.

[9] Lauren Mitchell and Elbert Huang, Diffusion and Disparities: Rural Uptake of Continuous Glucose Monitors, 47 Diabetes Care 344 (2024), https://diabetesjournals.org/care/article/47/3/344/154266/Diffusion-and-Disparities-Rural-Uptake-of.

"interoperable automated glycemic controller" specifically to describe AID system algorithms.[10] Insulin pumps and CGMs have been covered and paid by Medicare since 1999 and 2017 respectively, while the "brain" of the AID system is not separately paid under Medicare. To promote innovation and patient access to AID systems, CMS must ensure there is a payment pathway for the software or algorithm in an AID system. Software or algorithms in an AID system do not need a separate benefit category and can be appropriately categorized as a supply to DME. By doing so, CMS can promote access to AID systems as evidence becomes even more clear that AID systems improve diabetes management.

Given the clinical evidence to support the use of AID systems, we believe that the algorithm or software that drives an AID system meets the threshold as a "reasonable and necessary" item "for the diagnosis or treatment of illness or injury or to improve the functioning of a malformed body member"[11] to warrant Medicare coverage under the *Social Security Act* (SSA) for individuals with diabetes. Given that threshold requirement, DTAC believes that the software or algorithm that underlies the AID system can be considered a supply to DME. Regulation defines DME as an item that can withstand repeated use, has an expected life of at least three years, is primarily and customarily used for a medical purpose, is generally not useful to someone in the absence of an illness or injury, and is appropriate for use in the home.[12] Medicare currently covers both CGMs and insulin pumps as DME, meaning both components of an AID system, independently, are DME. Therefore, the complete AID system should also be categorized as DME. A supply to a DME is an item that is necessary for the effective use of DME.[13] The software or algorithm of an AID system is essential for the function of the entire AID system, as without it, the individual DME components do not operate together as part of the system. Under these criteria, nothing in statute or regulation would prevent the agency from classifying the software or algorithm of an AID system as a supply to a DME, thus allowing the agency to complete this classification within a short period of time.

By designating such software or algorithm as a supply to DME, CMS can provide needed recognition of the value and investment needed to innovate new and improved AID systems, which in turn will support continued beneficiary access to them. Since the software or algorithm of an AID system do not need a separate benefit category to be considered a supply to DME, CMS should make this reclassification as soon as possible. Finally, we reiterate that CMS must also enhance coverage for the other two critical components of AID systems, in part, by acting on the long pending NCD reconsideration request for Part B-covered insulin pumps and by updating, through the DME MACs, the LCDs for CGMs in alignment with the reconsideration request submitted on February 28, 2025.

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Thank you for the opportunity to provide comments to this RFI. Please feel free to contact Brian Lee at brian.lee@alston.com should you have any questions or if there are more details we can provide.

Sincerely,

Timothy P. Trysla
Executive Director
Diabetes Technology Access Coalition

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[10] 21 CFR 862.1356.
[11] 42 USC 1395y(a)(1)(A).
[12] 42 CFR 414.202.
[13] *Section 110.3 – Coverage of Supplies and Accessories, Chapter 15 – Covered Medical and Other Health Services – Medicare Benefit Policy Manual,* Centers for Medicare & Medicaid Services (Jan. 15, 2025), https://www.cms.gov/regulations-and-guidance/guidance/manuals/downloads/bp102c15.pdf.